

Arthur Davies

Age : 62 Years

Sex : Male



PID : 441

Apt ID : 2025441

Ref. By : Dr. Emily Thorne

Study Date & Time:

11:32 AM 15 Sep, 2026

Verified Date & Time:

11:58 AM 15 Sep, 2026

LUNG MRI (THORACIC MRI)

Lung MRI - Axial T2 & Diffusion Weighted (DWI) View

- Tumor exhibits irregular, spiculated margins with peripheral ground-glass halo and visceral pleural abutting.
- Direct bronchial bundling with abrupt termination and subsegmental bronchial cut-off sign.
- Prominent ipsilateral right hilar lymphadenopathy (short axis 1.4 cm), indicative of N1 nodal involvement.
- Contralateral left lung parenchyma, carina, and trachea lumen demonstrate normal patency.
- No pericardial invasion, pleural effusion, or chest wall osseous destruction identified on this sequence.
- Normal visualized anatomical structures.

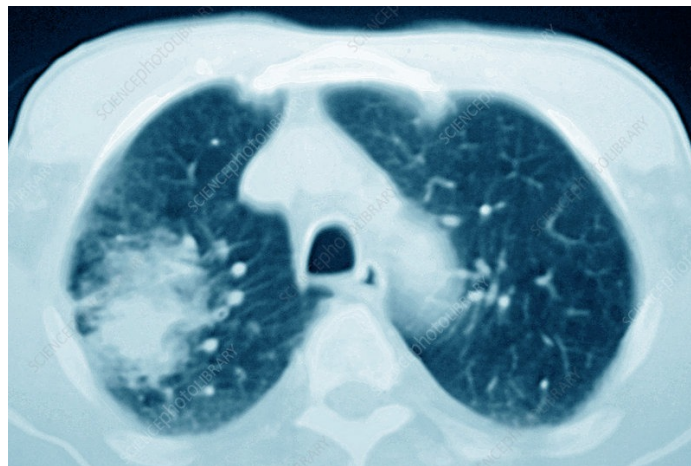
DISEASE IDENTIFIED:	Primary Bronchogenic Malignancy (Lung Tumor)
TUMOR / SUB-TYPE:	Non-Small Cell Lung Carcinoma (NSCLC) - High suspicion for Invasive Adenocarcinoma
STAGE / PHASE:	Locally Advanced Phase (Clinical Stage T2b N1 M0 - Stage IIB/IIIA)

IMPRESSION

Right lung spiculated parenchymal mass with regional bronchial obstruction and ipsilateral hilar lymphadenopathy, highly suspicious for Non-Small Cell Lung Carcinoma (NSCLC / Invasive Adenocarcinoma), Locally Advanced Phase (Stage IIB/IIIA).

ADVICE

CT-guided core needle biopsy / EBUS-TBNA for histopathological confirmation and biomarker assay (EGFR, ALK, PD-L1); whole-body PET-CT for M-staging.



[✓] DIGITALLY VERIFIED & APPROVED BY RADIOLOGIST

STATUS: OFFICIAL RECORD • VERIFIED AT 11:58 AM 15 Sep, 2026

Thanks for Reference

****End of Report****

Radiologic Technologists

(MSC, PGDM)

Generated on : 15 Sep, 2026 11:58 AM

Dr. Payal Shah

(MD, Radiologist)

Dr. Vimal Shah

(MD, Radiologist)

Page 1 of 1